

SUPRIM SUSPENSION

VISUP08-var1 (MY)

DESCRIPTION

White, viscous suspension with raspberry flavour.

COMPOSITION

Sulphamethoxazole 200 mg and trimethoprim 40 mg/5 ml suspension.

PHARMACODYNAMICS AND PHARMACOKINETICS

Sulphamethoxazole and trimethoprim, both broad-spectrum bacteriostatic anti-infectives, inhibit the bacterial enzymes, dihydropteroate synthetase and dihydrofolic acid reductase respectively. Their synergistic effect is attributed to the inhibition of tetrahydrofolic acid production, a cofactor for the synthesis of purines, thymidine and DNA, at two sequential steps in the folic acid biosynthesis. Sulphamethoxazole and trimethoprim, both undergo hepatic metabolism and renal excretion.

INDICATIONS

For treatment of :

- Urinary tract infections caused by *E. coli*, *Klebsiella*, *Enterobacter*, *P. mirabilis*, *P. vulgaris* and *Morganella morganii*.
- Acute otitis media in children caused by *H. influenzae* and *Streptococcus pneumoniae*.
- Acute exacerbations of chronic bronchitis in adults caused by *H. influenzae* and *Streptococcus pneumoniae*.
- Enteritis caused by *Shigella flexneri* and *S. sonnei*.
- Pneumonitis caused by *Pneumocystis carinii*.

CONTRAINDICATIONS

- Avoid in patients known to be hypersensitive to sulphonamides, frusemide, sulfones, thiazide diuretics, sulfonylureas or carbonic anhydrase inhibitors.
- Not recommended in patients with megaloblastic anaemia.
- Not recommended during pregnancy and lactation.

PRECAUTIONS

- Caution in patients with glucose-6-phosphate dehydrogenase (G6PD) deficiency, porphyria, renal and hepatic function impairment and in nursing mothers.
- All patients receiving prolonged treatment should be given regular blood examinations.
- Adequate fluid intake will help to prevent risk of crystalluria.
- There is an increased risk of adverse reactions in elderly patients in particular blood disorders and adverse renal/hepatic reactions.

WARNING

Respiratory toxicity

Very rare, severe cases of respiratory toxicity, sometimes progressing to Acute Respiratory Distress Syndrome (ARDS), have been reported during co-trimoxazole treatment. The onset of pulmonary signs such as cough, fever and dyspnoea in association with radiological signs of pulmonary infiltrates, and deterioration in pulmonary function may be preliminary signs of ARDS. In such circumstances, co-trimoxazole should be discontinued and appropriate treatment given.

Fatalities associated with the administration of sulphonamides and trimethoprim, either alone or in combination, have occurred due to severe reactions, including Stevens-Johnson syndrome, toxic epidermal necrolysis and other reactions. The drug should be discontinued at the first appearance of skin rash or any sign of adverse reactions.

Circulatory shock

Cases of circulatory shock, often accompanied by fever and not responding to standard treatment for hypersensitivity, have been reported with sulfamethoxazole + trimethoprim, mainly in immunocompromised patients.

MAIN SIDE/ADVERSE EFFECTS

Increased sensitivity of skin to sunlight, itching, headache, unusual taste in mouth, Stevens-Johnson syndrome, Lyell's syndrome, blood dyscrasias, hypersensitivity, hepatitis, methemoglobinemia, sore throat, bluish fingernails, lips and skin, blood in urine, lower back pain, crystalluria, thyroid function disturbance, and circulatory shock (vascular disorder with unknown frequency)

DRUG INTERACTIONS

- Concurrent therapy with the following drugs is not recommended:
Aminobenzoic acid (PABA), methenamine, penicillins.
- Concurrent therapy with the following drugs may necessitate dosage adjustments:
Oral anticoagulants, oral hypoglycaemics, methotrexate, phenytoin, thiopental and sulfinpyrazone.

OVERDOSAGE

Clinical features: Crystalluria, oliguria, anuria, bone marrow depression, gastrointestinal irritation and fatigue.

Treat overdosage by emesis or gastric lavage, if appropriate. Oral sodium bicarbonate and increasing fluid intake may help to eliminate sulphamethoxazole in the urine. Bone marrow depression due to trimethoprim may be treated with calcium folinate 3 - 6 mg IV, then 15 mg daily by mouth for 5 - 7 days.

DOSAGE AND ADMINISTRATION

This medication should preferably be taken with a full glass (240 ml) of water or on an empty stomach (either 1 hour before or 2 hours after meals).

Fluid intake should be sufficient to maintain urine output of at least 1200 to 1500 ml per day in adults.

Therapy should be continued for at least 10 to 14 days in acute exacerbation of chronic bronchitis; for 7 to 10 days in urinary tract infections; 5 days in shigellosis; 10 days in acute otitis media in children; 14 days in Pneumocystis carinii pneumonitis.

Adults

Antibacterial systemic: Oral, 800 mg of sulphamethoxazole and 160 mg of trimethoprim every twelve hours, or as directed.

Children

Antibacterial systemic:

- Infants and children up to 40 kg of body weight:
Oral, 20 mg of sulphamethoxazole and 4 mg of trimethoprim per kg of body weight every twelve hours; or as directed.
- Children 40 kg of body weight and over: See adult dose.

Note: Infants under 2 months of age - Use is contraindicated since sulfonamides may cause kernicterus in neonates.

Storage:

Store below 30°C. Protect from light and moisture.

Presentation/Packing:

Suspension x 60 ml, 100 ml, 3.6 Litres.

(Not all presentations may be available locally)

Manufactured by/Product Registration Holder:

HOVID Bhd.

121, Jalan Tunku Abdul Rahman
(Jalan Kuala Kangsar),
30010 Ipoh, Perak, Malaysia.

Revised date: January 2026